

Meeting Transcript — September 9, 2026

Participants: Isak Gutierrez, Thomas Abshier, Joelle Ater **Duration:** ~2 hours (7:28 AM – ~8:50 AM)

Part 1 — Steve Blank Book & Sales Strategy (~7:35–7:45)

Thomas: So I came up with an idea. We've got 10 machines we don't know what to do with. I was listening to the Steve Blank book your dad recommended — the Startup Owner's Manual. The thing that was very clear is: before you try to expand and do anything, you find out what sells. Find out what works.

We could get a hold of lots of doctors and tell them: "We've got this machine, it's not Medicare approved yet, we're working on it. It's an amazing thing that will help people. We're trying to find out if you as doctors would prescribe something like this."

If they aren't inclined, then we're barking up the wrong tree. If they say "that is something," then it's obviously worth pursuing the DME approach.

There's apparently a couple tracks. One is buying a DME — between \$1 and \$1.5 million if it's a mature one. Around \$100,000 to \$400,000 if they already have a business but don't have the 36 months. Or we can apply for our own.

I'm spending \$16,000 a month to support the staff. That's \$192,000 in a year. So we'll spend \$200,000 in a year just doing what we're doing now.

Part 2 — Joelle Joins, Strategy Discussion (~7:41–8:00)

Joelle: Sorry for the delay. I had to leave the house, so I'm just out walking.

Thomas: (*continues*) The question is: is applying for the DME and spending a couple hundred thousand dollars, then using that year to do market research — is that the right move? The Steve Blank book said: the big message is you've got to find your market. Maybe pre-selling it. We could advertise and say "we will have this available via Medicare in a year, if you're interested, let us know."

Joelle: My concern would be: we do a bunch of pre-sales and then really it's like you're selling the idea, and they find something else in the meantime. That's what I would do personally. Unless it was something that just — do we stand out enough for someone to wait a year?

Thomas: No, the answer is no, because everybody else is shinier. So we say "would you, if you had Medicare available, would you do this?" They say absolutely. "We don't have that for a year, but we do have some available now. Would you be interested in buying from us now at \$12,000? Here's our financing package."

Joelle: I like that possibility a lot better — having the potential to sell right now while we develop. What spurred the idea to become a DME? You said it's hundreds of thousands?

Thomas: \$1.2 million to buy one that's been in business for 36 months. If you want one that somebody already has set up but doesn't have the 36 months, then \$100,000 to \$400,000, but you don't have Medicare availability for another year because it only transfers if it's over 36 months.

Joelle: What about just getting connections to existing DMEs?

Thomas: That was the problem — they're going to take a big chunk of revenue and we end up with a short margin.

Isak: Standard is between 20% and 40%, probably leaning 30 to 40. At the lowest, \$2,000 per walker, at the highest, \$4,000 to \$5,000 if we're selling at \$16,000.

Joelle: That's still doable in the meantime, right? Like, that's something we could do at first while we figure out if we want to be our own DME?

Isak: Yeah, totally.

Thomas: The concern is we'd be educating the competition. If we pull out and say "we've got our own DME now," they'll go find Mobility Research to cut a deal.

Joelle: What if we did both? What if the other DMEs carried our products as well?

Thomas: The cost is the 20-40% profit loss.

Joelle: It's like how people sell their music in Walmart even though it's at a loss — to get it out there.

Part 3 — Pricing & Buyback Program (~8:12–8:33)

Thomas: 12,000 is possible because other people are in the market for 12. 75% of 12,000 is \$9,000. How much is the DME going to want? If 30%, that's \$2,700. So we'd get \$6,300.

Isak: If we're going through another DME, we wholesale at \$6,000 and make about \$2,500 after COGS.

Thomas: So we could sell it for \$12,000 and be competitive. If we have our own DME, we'd make \$6,300 per unit.

Joelle: What if we had a buyback program so they could eventually get it prescribed? So it'd be like a no-loss to get it right now.

Thomas: Oh my gosh, that is a totally new concept. We refurbish the returned units, give them a new prescribed unit through Medicare. We just have a buyback program — if you ever get this by Medicare, you'll be able to get it by prescription.

Joelle: And selling refurbished to those other outliers.

Thomas: We could buy it back for \$6,000 wholesale. With our own DME, we can sell the refurb for \$9,000.

Joelle: The only problem is I wouldn't want to guarantee the buyback timeline since we don't know when Medicare would be available.

Thomas: We'd say we're pursuing it. We can't guarantee it. All we can tell you is we're working on it, and you can talk to your doctor.

Part 4 — Market Testing & Sales Channels (~8:00–8:10)

Joelle: I have a few connections. I help a nonprofit called Riley's Residence — we're building a property with multiple houses for people with intellectual and developmental disabilities, with a therapy center in the middle. Because we've been starting this, we have all these connections to others doing similar things. I'd like to start with that market.

I also have one of my uncles in Spokane — he runs a caregiving service, like a care.com kind of thing. He's also been in pharmaceutical sales, so he has the experience of talking to doctors.

Thomas: Your idea of having a lot of different markets, everybody take a different one, and then we share what we learned — that would be a huge learning exercise.

Joelle: I have a chiropractor friend that specifically sees the special needs and autistic community, people that are wheelchair-bound.

Thomas: The OTs and PTs are the people most likely to refer. There's a group of people that have at least prescribed wheelchairs — I don't know what medical specialty that is.

Joelle: My friend who started Riley's Residence — she has to take her son, who's been wheelchair-bound since he was vaccinated, to a doctor every single year to get the wheelchair re-prescribed. That's because it's Medicare.

Thomas: Oh my gosh! That opens up an entirely new concept. That means if you're on Medicare and getting a wheelchair, you have to see a doctor every year. That means there's a type of doctor that sees that type of patient.

Joelle: Let me find out more about that process.

Part 5 — Immediate Action Plan (~8:35–8:50)

Joelle: When we get the notes from this meeting, we should have Claude give us an order. I can't do financing until we have insurance. Is insurance dependent on anything?

Isak: Insurance is not. Insurance is the next move.

Joelle: To sell right now, we'd need: insurance, financing, somewhere to direct them (website), videos and pictures.

Isak: I'll coordinate with Dad for a photo/video shoot. White background, the works. I also want to record him just talking about his experiences.

Joelle: Once we have media, I can create a pitch deck or one-pager to send out to doctors or facilities. CRM doesn't take long to set up. Is the bank account ready?

Isak: I had applied. They were asking for the partner agreement documents. I'll submit those and finish up.

Joelle: What about soft goods?

Isak: I asked Mom to do one full batch. I gave a wide estimate of about a week and a half per batch. I should probably drive over and help her set up the sewing area.

Joelle: I'll work on: logo, CRM, financing, and Stripe once we have bank info. What about the convenience fee for Stripe?

Thomas: We just pass that on to the customer.

Joelle: What this means is we could really start now.

Thomas: We could start now. We just need a finance company in place, insurance in place, and then hit the pavement.

Thomas: *(after Joelle left)* I think we should actually be pursuing the DME thing. Just start now and plug through it. Do our year and pay our dues. If we get to the end and it was useless, we keep it for 3 years and sell it for a million dollars. There will be no loss in it.

Isak: Not a bad investment.

Thomas: Have Claude run a cost for doing Medicare and for establishing a DME.

Isak: I'll make some calls, try to check all those boxes. I think we got back up to speed.

Thomas: Good. This is a reasonable plan. We're getting the bones of the situation. Then we can try and learn what about the structure needs to be added and refined.